



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ساميه محروسه محمد محمد

العنوان : 444

الحى / المدينة : الدقي

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 30-09-2026

رقم البطاقة : 27502062200364 رقم التليفون : 01159511676

العمر (تاريخ الميلاد) : 06-02-1975 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك الإخصائي الاجتماعي

المقر بما فيه التوقيع: (.....)

التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة. تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: سامية مخروسة محمد محمد
National ID: 27502062200364



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشان تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Mammography and breast ultrasound

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

سامية
محروس محمد محمد حسن
٥ بن احمد حبيب - بين السرايات
الدقي - الجيزة

١٩٧٥/٠١
٢٧٥٠٢٠٦٢٢٠٠٣٦٤
K03289740

Dms

تاموكسيفين

٢٠٢٣/٠٨ ٢٧٥٠٢٠٦٢٢٠٠٣٦٤
حاصلة على بكالوريوس خدمة اجتماعية

انثى مسلمة متروجة
اسماعيل خميس حسين حسن
البطاقة مصرية حتى ٢٠٢٠/٠٨/٣٠

١

Patient Name :	أحمد محمد يوسف	Request No. :	13003306
Patient No. :	27502062200364	Request Date :	20/08/2024
Path No. :	SR-1196/24	Verification Date :	08/09/2024
Sex :	Female	Financial Category :	أورام خارج المستشفى - فئة الأورام
Age :	49 Y	Department :	Out Patient
Referring Doc. :		IIIC No. :	
		Registered by :	Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass underwent WLE & axillary clearance.

Nature of Specimen: Spec revision.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9)

Results:

- IPT (Positive control showed appropriate reactivity).
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Right breast mass, Slide revision:
invasive duct carcinoma, Grade II,
Positive lymph nodes (2/20)

PT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

Scoring system: 'ER, PR'

Verified by :

Patient Name: **أ.م.م. هبة محمد حسن**
Patient No: **27502062200364**
Path No: **SR-1186/24**
Sex: **Female**
Age: **49 Y**
Referring Doc:

Request No: **13003306**
Request Date: **20/08/2024**
Verification Date: **08/09/2024**
Financial Category: **أورام النسائية - عيادة**
Department: **Out Patient**

Registered by: **Ali mostafa**

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: *Her2*

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Amal Hareedy.
Dr. Heba Hassan.

Verified by:

Printing date: 08/09/2024 11:42:13

Page 2 of 2

Printed By: marwa mhammed youssef



Name: Mrs. Samya Malrous Mohamed Mohamed

Date: 28/11/2023

ID:1123033152

Acc.:291100725

Referring: Hospital El-Sholkh Zayed Specialized

CT scan examination of the chest with IV contrast revealed:

- Clear both lung parenchyma with no pulmonary masses, calcification nor cavitation.
 - No pleural or pericardial collection.
 - No hilar or mediastinal masses.
 - Heart and great vessels appear normal.
 - The upper section of the abdomen shows no abnormality.
 - No pathologically enhanced focal lesions or cysts seen.
- Conclusion:**

Conclusion:

Normal GT chest study.

2501 22nd St., Co. REC.
 2nd St., S.W. 10th St. 10000

Truly yours;
Dr. Mourad Salama, FEBR, ABRI



14/04/2026
14/04/2026
14/04/2026

Patient's Name:

SSAMIA MAHROUS

22/04/26

Date:

2026-04-01

Referred by:

Prof.Dr.

Hospital:

Goustave Roussey International Egypt



Bilateral high resolution breast sonography revealed:

Clinical summary: Status post right CBS & ALAND, followed by radio therapy, routine follow up study.

Findings:

Right breast.

- Preserved fibroglandular densities.
- UOQ area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
- Thickened skin and coarsen trabeculae.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Preserved fibroglandular densities.
- No solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 2): Post-Therapeutic sequelae.

Left breast (BIRADS 1): Normal study

Recommendation: -

For annual follow up.

Best Regards,

DR. Amal Hegazi, M.Sc.

Gluss, V. - 1/4/2026

NAME: samia mahros

DATE: 1/4/2026

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- AVF, average size uterus measuring about: 7x4x3cm in its longitudinal, transverse & AP diameters respectively.
- The myometrium shows homogenous texture with no focal lesion
- The endometrial line is 3mm.

○ **Ovaries: -**

- Both ovaries show no focal lesion
- No gross adnexal soft tissue masses or cysts.
- No free fluid in the Douglas pouch.

Conclusion:

Unremarkable study

Best regards
Dr. NERMEEN RAMADAN FEBR, MD

المرضى، ص. البنية والجهاز الهضمي

NAME: samia mahros

DATE: 1/4/2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (15cm at Rt MCL) homogenous parenchymal echo pattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- . The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Unremarkable study

Best regards

Dr. NERMEEN RAMADAN FEBR, MD

ECHO REPORT

NAME: سامية حوروس مكيه DATE: 22/10/2025
ID NUMBER: 2018 / 22 SEX: F AGE: 50

EF	70%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVESD	2.8	2-4cm	LA	3.6	2-4cm
FS	31%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function Normal
SWMA No RWMA
LA and Ao Root Dimensions Normal

Mitral Valve Normal
Aortic Valve Normal
Tricuspid Valve Normal
Pulmonary Valve Normal

Right Side and Systolic Function TAPSE: 2.3
Intracardiac Masses and Effusion

DR. 

Patient Name: Samia Mahrous Mohamed Mohamed Hassan	Date : 07/09/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 50 Year(s)
Medical ID: 2708/2024	Pat. ID: 90761

HIGH FIELD MRI OF THE BRAIN WITH CONTRAST

findings:

Few Foci of altered signal intensity are seen in both frontal regions displaying high T2 & FLAIR signal

intensities

Normal size and configuration of the ventricular system with no evidence of deformity or displacement.

No brain stem areas of abnormal signal intensity

No mid line shift.

No extra-axial collections.

No abnormally enhancing lesions .

OPINION:

- Bilateral frontal foci of altered SI ? lacunar infarcts ? otherwise for clinical correlation

Dr. Reham Khedr (MD)

Patient Name: Samia Mahrous Mohamed Mohamed Hassan	Date : 15/10/2024
Exam : C.T ABDOMEN & PELVIS	Age : 49 Year(s)
Medical ID: 2708/2024	Pat. ID: 90761

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of right CBS.

I Chest:

- *Right breasts sin thickening is seen.*
- *Right axillary scarring, no pathologically enlarged lymph nodes.*
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Average sized liver showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Few tiny lucent foci of both iliac bones.

Opinion:

- *No right breast gross masses on non-contrast basis.*
- *No sizable pulmonary or hepatic deposits on non-contrast basis.*
- *Few tiny lucent foci of iliac bones, possibility of benign nature for bone scan correlation*

Dr. Ali Elsisy, MSc.

Signature Prof. Menna Tallah Hanafy

23/10/2024-09:15


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية

ساميه
 محروس محمد محمد حسن
 هـ ش أحمد حبيب - بين السرايات
 الدقي - الجيزة

١٩٧٥/٠٩/١٦
 ٢٧٥٠٢٠٦٢٢٠٠٣٦٤
 KD3289740


 ٢٠٢٢/٠٨/٢٧
 ٢٧٥٠٢٠٦٢٢٠٠٣٦٤
 حاصلة على بكالوريوس ختله اجتماعيه
 متروجه مسلمة
 اسماعيل خميس حسين حسين
 البطاقة سارية حتى ٢٠٣٠/٠٨/٣٠



Patient Name: Samia Mohamed Hassan	Mahrous Mohamed	Date : 29/10/2024
Exam : MAMMOGRAPHY BOTH		Age : 49 Year(s)
Medical ID: 2708/2024		Pat. ID: 90761

BILATERAL BREAST MAMMOGRAPHY AND HIGH-RESOLUTION ULTRASOUND

Indication:

Right breast conserving therapy

Report:

- Extremely dense fibroglandular parenchyma (ACR d)

Right breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
 - No suspicious mass lesions, micro calcifications
 - No sizable collection
 - Diffuse edema pattern in the form of increased trabeculations and skin thickness.
 - No regional lymphadenopathy
- Left Breast:**
- No suspicious micro calcifications or architectural distortion detected.
 - No spiculated mass
 - Normal retro-areola ducts
 - Normal skin thickness, and nipple-areola complex
 - Nonspecific axillary LN.

Opinion:

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Best regards,

Dr Khafoud Khaleel

Dr. Amira K. Radwan MD,

Breast Imaging consultant,

WCL Cairo University

29/10/2024 3:37 PM

Patient Name: Samia Mahrous Mohamed Mohamed Hassan	Date : 15/10/2024
Exam : C.T ABDOMEN & PELVIS	Age : 49 Year(s)
Medical ID: 2708/2024	Pat. ID: 90761

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS:**

- Reported case of right CBS.

I Chest:

- *Right breasts sin thickening is seen.*
- *Right axillary scarring, no pathologically enlarged lymph nodes.*
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Average sized liver showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Few tiny lucent foci of both iliac bones.

Opinion:

- *No right breast gross masses on non-contrast basis.*
- *No sizable pulmonary or hepatic deposits on non-contrast basis.*
- *Few tiny lucent foci of iliac bones, possibility of benign nature for bone scan correlation*

Dr. Ali Eisisy, MSc.

Signature Prof. Menna Tallah Hanafy

23/10/2024 08:15

906 Jaf

Date : 05/10/2025	Patient Name: Samia Mahrous Mohamed Mohamed Hassan	
Age : 50 Year(s)	Exam : MAMMOGRAPHY BOTH	
Pat. ID: 90761	Medical ID: 2708/2024	

Bilateral Digital soft tissue mammography with high resolution

breast sonography revealed:

Clinical summary: Post Right CBS and ALAND followed by radiotherapy, routine follow up study.

Findings:

Right breast.

- Heterogeneously dense breast . (ACR c)
- Thickened skin and coarsen trabeculae.
- architectural distortion with no speculations, masses, asymmetry , or suspicious microcalcifications.
- **Ultrasound** demonstrated para areolar , LQ area of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Heterogeneously dense breast . (ACR c)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

- **Right breast (BIRADS 2): Post-therapeutic sequelae.**
- **Left breast (BIRADS 1): Normal study.**

Recommendation:-

- **For annual follow up.**

Best Regards,
DR. Rania Ahmed, MD

Signature Dr. Amal Hegazy
05/10/2025 12:47 PM

أحمد
أحمد



Patient Name: Samia Mahrous Mohamed Mohamed Hassan	Date : 29/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 49 Year(s)
Medical ID: 2708/2024	Pat. ID: 90761

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Right breast conserving therapy

Report:

- Extremely dense fibroglandular parenchyma (ACR d)

Right breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Best regards,

Dr Kholoud Khaled

Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
29/10/2024 3:37 PM

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمي

The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital



Patient Name: Samia Mahrous Mohamed Mohamed
Date: 18/6/2025

مراجعة 18/6/2025

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

- Status post right CBS 1.5 year ago on hormonal therapy.

Technique:

- CC and MLO views.
- High resolution ultrasound.

RIGHT BREAST:

- Diffuse edema pattern with increased skin thickness and coarse trabeculation.
- Operative bed showed parenchymal distortion and scarring, no internal vasc.
- No sizeable suspicious masses or collections.
- No sonographic evidence of regional pathological lymph-nodes.

LEFT BREAST:

- Extremely dense fibroglandular parenchyma (ACR d).
- No spiculated masses or grouped micro-calcifications.
- Complementary ultrasound showed homogenous parenchymal texture.
- No solid or cystic masses are detected.
- Normal skin thickness & nipple/areola complexes.
- Non specific axillary lymphnodes.

Conclusion:

- Status post right CBS showing diffuse edema pattern with parenchymal distortion and scarring likely post therapeutic changes and no evidence of suspicious masses or collections.
- Left breast benign findings.
- (BIRADS-3)

Recommendations:

Follow up after 6 months.

Muti Obisgeef
Dr. Yasmine Ibrahim

Nuclear Medicine Unit

Patient Name: Samiah Mahrousa
Mohamed

Date: 5/1/2025

Referring physician: ^{99m}Tc-MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No scintigraphic evidence of bony metastases.
- Heterogenous abnormal radiotracer uptake is seen at the spine, more determined at, mid dorsal, lumbar vertebrae, likely of spondylodegenerative changes,
- Osteoarthritic changes are seen at, both shoulders,
- Both kidneys are normally seen.

Conclusion:

No definite hot spot that could account for osseous metastatic lesions

Thanks for the referral

Nuclear Medicine Unit

DR. Sherine Mady, MD

DR/ Mohamed Emam

Patient Name	الاستاذة / سامية محروس محمد محمد	Request No.	: 13003306
Patient No.	: 27502062200364	Request Date	: 20/08/2024
Path No.	: SR-1196/24	Verification Date	: 08/09/2024
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 49 Y	Department	: Out Patient
Referreing Doc. :		IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass underwent WLE & axillary clearance.
Nature of Specimen: Slide revision.

Technique#

Sections were prepared from the paraffin block, processed in Bench-Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: No membrane reaction in tumor-cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Right breast mass, Slide revision:
invasive duct carcinoma, Grade II .
Positive lymph nodes (2/20).

PT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

Scoring system: "ER, PR"

Verified by :

Patient Name	: سامية محروس محمد ممت	الأستاذة	Request No.	: 130033306
Patient No.	: 27502062200364		Request Date	: 20/08/2024
Path No.	: SR-1196/24		Verification Date	: 02/09/2024
Sex	: Female		Financial Category	: نفقة الدولة - أورام دار السلام
Age	: 49 Y		Department	: Out Patient
Referreing Doc :			Registered by	: Ali mostafa

Pathology Report

Clinical Data: Right breast mass underwent WLE & axillary clearance.
Nature of Specimen: Slide Revision.

Gross Examination:

- Six paraffin blocks labeled by (2509) and 6 slides were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (2+2+2) Grade II.
- * Ductal carcinoma in situ: 30%.
- * Perineural invasion: Not present.
- * Angioinvasion: Not present.
- * Microcalcifications: Not present.
- * Lymph nodes: Positive lymph node (2/20).

Diagnosis:

Right breast mass, underwent WLE & axillary clearance, Slide Revision:
Invasive duct carcinoma, Grade II.
- Positive lymph nodes (2/20).
- For further IPT (ER, PR, Her2 & Ki-67).

Signature

Prof. Dr. Amal Ahmed Hareedy.
Dr. Heba Hasan.
Dr. Rufaida Abdullah.

Verified by :

Patient Name
Patient No.
Path No.
Sex
Age
Referring Doc.

الأستاذ / سامية محروس محمد
27502062200364
SR-1196/24
Female
49 Y

Request No. 13003306
Request Date 20/08/2024
Verification Date 08/09/2024
Financial Category أورام دار السلام - نفقة الدولة
Department Out Patient

Registered by Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

• Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Amal Hareedy.
Dr. Heba Hassan.

Verified by :

Printing Date : 08/09/2024 11:42:13

Page 2 of 2

Printed By : .marwa mohamed youss

CAP

Patient Name: Samia Mahrous Mohamed Mohamed Hassan

Exam : ultrasound ABDOMEN& PELVIS
Medical ID: 2708/2024

Date : 30/06/2025

Age : 50 Year(s)
Pat. ID: 90761

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (15cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching
- Uterus and adnexal:** no abnormalities could be detected

Opinion:

Unremarkable study

Signature Dr . Nermien Ramadan

30/06/2025 11:52 AM

Patient Name: Samia Mahrous Mohamed Mohamed Hassan	Date : 07/09/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 50 Year(s)
Medical ID: 2708/2024	Pat. ID: 90761

HIGH FIELD MRI OF THE BRAIN WITH CONTRAST

Findings:

- Few Foci of altered signal intensity are seen in both frontal regions displaying high T2 & FLAIR signal

intensities

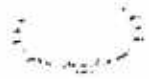
- Normal size and configuration of the ventricular system with no evidence of deformity or displacement.
- No brain stem areas of abnormal signal intensity
- No mid line shift.
- No extra-axial collections.
- No abnormally enhancing lesions .

OPINION:

- **Bilateral frontal foci of altered SI ? lacunar infarcts ? otherwise for clinical correlation**

Dr. Reham Khedr (MD)

Signature Reeham.Khader
09/09/2025 3:59 AM



Patient Name	: الأستاذ / ساميه محروس محمد محمد	Request No.	: 13003306
Patient No.	: 27502062200364	Request Date	: 20/08/2024
Path No.	: SR-1196/24	Verification Date	: 08/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 49 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass underwent WLE & axillary clearance.

Nature of Specimen: Slide revision.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Right breast mass, Slide revision:

Invasive duct carcinoma, Grade II.

Positive lymph nodes (2/20).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"

Verified by :



د / منعم شريف
إستشاري الباثولوجي
أستاذ بكلية الطب جامعة القاهرة (م)

د / أحمد نعيم
إستشاري الباثولوجي
أستاذ بكلية الطب جامعة القاهرة (م)

د / سميرة عبد اللطيف
إستشاري الباثولوجي
أستاذ بكلية الطب جامعة القاهرة (م)

Name	سامية محروس محمد	Age	48Y	Code	2509 - 23
Referred by	الد. وائل عبد السلام	Date	24/10/2023	Sex	Female
Clinical data	Right breast malignant lesion				
Specimen	1.Right breast lesion with safety margin 2.Right axillary lymph nodes				

PATHOLOGY REPORT

GROSS: Two specimens received:

1. Fibrofatty tissue piece; measures 6x4.5x4cm, margined by skin ellipse; 6x3.5cm. Serial sections revealed an ill-defined mass; 3.5x2cm, located 0.5cm from least margin.
2. Axillary dissection; fibrofatty tissue entangling; 18 lymph nodes; up to 1cm in diameter.

1.2 mass 2. margins 4.5.6 L4s

MICROSCOPIC PICTURE: Sections examined from the detected mass revealed a malignant growth composed of tubular structures & cords of malignant ductal cells showing moderate grade of cellular anaplasia, with desmoplastic stroma. Duct carcinoma in situ (cribriform & necrotizing) was seen representing about 40% of the growth bulk. Lymphovascular tumor emboli and perineural invasion were not evident. Section examined from resection margins was free from tumor infiltration. Sections examined from the dissected axillary lymph nodes show metastatic tumor deposits in 2 lymph nodes with no capsular invasion (2/18).

DIAGNOSIS: 1.Right breast lesion with safety margin 2.Right axillary lymph nodes:

- Invasive Duct Carcinoma (N&T), Grade II, associated with 40% Intermediate Grade DCIS
- Free And Adequate Resection Margins.
- Positive Axillary Lymph Nodes (2/18) With no Extra- capsular Invasion.
- Stage: pT2, N1.

Prof. Dr. Sonia Soliman; MD

Prof. Dr. Ahmed Naeem; MD

Prof. Dr. Mohamed Sherif; MD

Sonia Soliman

A. Naeem

M. Sherif

9 برج تولوة فيصل - شارع فيصل الرئيسي - امام كلية التربية الرياضية - الدور الثاني

9 Faisal - Near Tolwa - front of Faculty of Physical Education - second floor

☎ 011 490121 - 023853601

☎ 011 43299928 - 0230937015

Pathology Report

[[DOCTYPE:PATHOLOGY_REPORT]]



Presidential Program for Women's Health
Ministry of Health and Population

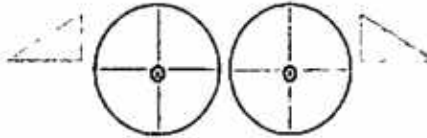
البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID: 1- Name : سلمية محروس محمد محمد Sex: Female 3- Medical number: 83225979 Old Med : Old Med :

2-ID number: Patient Age : 50 Y



Family history	Family History	Phone	Phone	BMI	BMI	REASON OF MDT	REASON OF MDT
			01159511676				NEW
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT
							17/09/2024 ☐
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :	Mets Workup :
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BX IDC BX COMMENT:	ER 8/8 ER COMMENT:	PR 7/8 PR COMMENT:	Her 2 Neu 0 Her 2 Neu COMMENT:	SISH SISH COMMENT :	Ki67 10
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T p2	N p1	M 0
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Sonomammography

PATIENT HISTORY
upfront It BCS 10-2023 OUTSIDE THEN FINISHED 4 AC AND 12 WEEK TAXOL
PT IS ON PORT

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
NOLVADEX for 5 years
THEN AI for 5 years
ZOMETA every 6 months

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ شريف مسعود د/ يحيى اسماعيل	RADIOLOGY	RADIOLOGY د/ شريف صلاح
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Patient Name	: الأستاذ / سامية محروس محمد محمد	Request No.	: 13003306
Patient No.	: 27502062200364	Request Date	: 20/08/2024
Path No.	: SR-1196/24	Verification Date	: 02/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 49 Y	Department	: Out Patient
Referring Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Right breast mass underwent WLE & axillary clearance.
Nature of Specimen: Slide Revision.

Gross Examination:

- Six paraffin blocks labeled by (2509) and 6 slides were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (2+2+2) Grade II.
- * Ductal carcinoma in situ: 30%.
- * Perineural invasion: Not present.
- * Angioinvasion: Not present.
- * Microcalcifications: Not present.
- * Lymph nodes: Positive lymph node (2/20).

Diagnosis:

Right breast mass, underwent WLE & axillary clearance, Slide Revision:
Invasive duct carcinoma, Grade II.
- Positive lymph nodes (2/20).
- For further IPT (ER, PR, Her2 & Ki-67).

Signature

Prof. Dr. Amal Ahmed Hareedy.
Dr. Heba Hasan.
Dr. Rufaida Abdullah.

Verified by :

